A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

30 April 2026

Dr Mary Ann Johnson
Epworth Eastern

Dear Dr Johnson,

Re: Mr Craig Parkin
DOB: 31/03/1981 Phone: 0417 854 115
Address: 13/8 Beavis Street, ELSTERNWICK, VIC, 3185
Email: cd_parkin@yahoo.com.au
Medicare: 5110780427 / 1

Summary

- **Post-fundoplication dysphagia and odynophagia with fluctuating symptom severity.**
- **Trial of budesonide slurry to address possible oesophageal inflammation.**
- **Continuation of amitriptyline at 25mg and re-trial of nizatidine.**
- **Ongoing consideration for fundoplication reversal after the twelve-month post-operative mark.**
- **No action required by you.**

I had the pleasure of conducting a telehealth review with Craig Parkin regarding his ongoing gastrointestinal symptoms following his fundoplication. Craig reports that his symptoms remain quite variable, characterised by a "roller coaster" of progress. In March, he experienced a significant flare of odynophagia, described as a pinching or cutting sensation in the oesophagus, alongside intermittent dysphagia where food felt trapped in his chest. These symptoms appear to be most prominent in the late afternoon and evening following his main meal.

He noted that this recent exacerbation coincided with a viral illness and a period of increased psychosocial stress. While there has been some mild improvement in the last fortnight, his quality of life remains significantly impacted compared to his pre-operative state.

Currently, Craig is taking 25mg of amitriptyline daily. He notes some benefit but reports associated lethargy and increased sleep requirements. He is also taking domperidone and a low dose of lansoprazole. We discussed his previous trial of nizatidine, which he had ceased due to headaches; however, he is open to attempting a re-trial to see if it assists with his current symptom profile.

Impression and Plan

Craig remains frustrated with the outcome of his fundoplication and is strongly considering a surgical reversal. I understand he is in active talks with you to consider a reversal at the end of the year. I have encouraged him to continue the amitriptyline in the interim, as it may serve to blunt the sensory severity of his symptoms both now and through any potential revisional period.

Given Craig previously noted a subjective improvement while taking prednisolone, I have prescribed a trial of budesonide slurry. He has been instructed to mix the medication into a slurry with honey or stevia to coat the

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oesophagus, remaining nil by mouth for 30 minutes following administration. This aims to treat any underlying inflammatory component that may be contributing to his symptoms. We discussed the possibility of eosinophilic oesophagitis; however, given the low clinical probability and previously negative (though limited) biopsies, we will not pursue further gastroscopy at this stage. Craig will follow up with me on an as-needed basis following his upcoming surgical review and his overseas travel.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

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320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065